LOUISIANA
DEPARTMENT OF HEALTH
Office of Rural Health Transformation and Sustainability

Rural Health Transformation Program Updates

October 1, 2026



Stevens Amendment

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General Information

- Slides from presentation will be added to RHTP Website
- Questions can be entered into the Chat
- Website being updated regularly – check for updates
  - Surveys (Important that we hear from you!)
  - Important Dates Section

ORHTS New Team Members

RHTP	Health Care Access
Melder Burton, Program Manager	Anita Stewart, Health Care Access Director
Shelia Sterling, Program Manager	
Alex Levy, Budget Manager	

Budget Year 1 Timelines

Activity	Timeline
Notification of initial award selection for all strategies	Current - October 2nd
Notification of denial for all strategies	Current - October 16th
CEA / contract completion	Current – October 30th
Quarterly Report to CMS	November 29, 2026

Helpful Hints for Contracting

- Establish a SAM Unique ID registration
  - Set this up at sam.gov

- Establish/re-activate State Vendor number.
  - All vendors must review vendor accounts yearly for accuracy.
  - W9's must be submitted to OSRAP to unblock vendor accounts.
  - For questions regarding vendor accounts, reach out to doa-osrap-lagov@la.gov; 225-342-1097; Vendor Information - Louisiana Division of Administration

- Please allow 2 business days for responses to emails regarding contracting. We have a high influx of emails and we are working diligently to respond back to everyone.

Requirements for Budget

- LA Contract Budget Year 1 = 7/1/2026 – 9/30/2027
- Budget projections must be broken out monthly
- Administrative vs. programmatic costs
  - Administrative = costs that support recipient operations, administration, or oversight of program; purpose is to provide specific goods or services that support or facilitate the recipient's program operations (ex. Project management, data collection, compliance activities)
    - Subject to 10% administrative cost cap
    - Includes indirect and other costs
  - Programmatic = purpose is to implement and carry out program initiatives in partnership with recipient (not subject to 10% cap)
    - Must provide sufficient documentation to demonstrate purpose and function and how costs directly relate to program initiative

Requirements for Budget

- **Personnel** – list each staff member working on project, level of effort (%FTE), annual salary, number months on project, portion charged to grant. Justification with description of position, scope of responsibility, and programmatic (if applicable)
- **Fringe Benefit** – cost calculated as percentage of personnel, including rate used and basis for calculation
- **Travel** – All in-state and out-of-state, including mileage, airfare, lodging, and per diem
- **Equipment** – any single item costing $10,000 ore more with useful life more than one year, including number and cost per unit, with justification for why necessary and how costs determined
- **Supplies** – Items under $10,000, including number and cost per unit, with justification for why necessary
- **Contractual** – includes 2nd tier for contractors / consultants. Include budget, how costs determined and brief description of services
- **Other direct** – anything not fitting into above categories with number / cost per unit and justification
- **Indirect** – include rate used to determine costs

Helpful Hints for Planning for Implementation

- Once your Cooperative Endeavor Agreement is fully executed, a Program Manager will be assigned to your project.

- The Program Manager will contact you (on the phone or via email) to discuss:
  - Invoicing
  - Reporting
  - Challenges to completing the project plan as proposed
  - Continuation/New requests for future years

- The Program Manager will be your primary contact and will connect you with the appropriate team members, as needed.

Year 1 funding allocation by initiative ($208.4M total)

As of 9/4

WORKFORCE $50.5M 24%	TECH INNOVATION $58.0M 28%	SUST. ACCESS $42.1M 20%	MRAHA $26.0M 13%	INNOVATIVE CARE $30.0M 15%
Rural HERO Grants **$20.0M**	Net-New State EHR Instance **$32.9M**	Rural Health Facilities Capital Improvement **$41.6M**	Alternate PACE Sites **$6.3M**	Innovative Care Models (APM) **$20.0M**
Rural Clinician Credit Bank **$10.0M**	Rural Tech Catalyst Fund **$20.0M**		Partnerships with Behavioral Health **$5M**	Risk-Sharing Value-Based Arrangements **$10.0M**
Education / Training Pathways **$7.0M**	Interoperability **$1.6M**		Telehealth Infrastructure **$4.71M**	
Grow Your Own **$6.0M**	Digital Literacy Education **$3.5M**		Community Nutrition & Fitness **$3.76M**	
Community Paramedicine / Tele-EMS **$4.0M**			Regional Care Conveners **$3.50M**	
Collaborative Provider Models **$3.0M**			Food is Medicine **$2.70M**	
UL Lafayette CAII – RHTLA **$488K**				

ADMINISTRATIVE COSTS: $5.57M	Personnel + fringe: $2.09M	Indirect: $3.13M	Travel, supplies, other: $0.36M

What's Ahead?

- ORHTS team planning for Year 2
  - Streamline processes and application process
  - Webinars following each NOFO

- Anticipated timeline for Year 2 NOFO publication (January – March 2027)
  - 2027 Calendar of Events to be published

- Regional Visits
  - One region per month beginning in January
  - Town halls, site visits, regional meetings
  - 2027 Calendar of Events to be published

- Think tanks and round table meetings

Success Stories

- Tell us how this program is impacting your organization and community! Share you success stories with us via YOUR RURAL SUCCESS STORY HERE

Upcoming Engagement

- RHTP Advisory Council Meeting
  - November 5th, 9am – 12pm, Capitol, House Committee Room 3
  - Open to the public and streaming

- Next monthly webinar will be on Thursday, December 10, 2026
  - Time will change to 12:30pm – 1:30pm based on feedback

- For more information, reach out to Ruralhealthtransformation@la.gov

Office of Rural Health Transformation and Sustainability

Website:

https://ldh.la.gov/page/rural-health-transformation-program

Email:

RuralHealthTransformation@la.gov

Federal Contact:

MAHARural@cms.hhs.gov

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